

Project Abstract Summary

FAIRBANKS MEDICAL CENTRE LIMITED

CDC-RFA-JG-26-0054

Your health, our mission.

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Project title: Helping Uganda find and stop health threats earlier through FairBanks Community Reach, CHW and VHT reporting, and FCHIP tools that feed Ministry of Health and NISS pathways

Applicant: FAIRBANKS MEDICAL CENTRE LIMITED

Project director: Racheal Nabukeera, Managing Director and Co-founder

Period: 09/30/2026 to 09/29/2031

Funding type: Cooperative agreement | CFDA: 93.318

Year 1 ask (draft): \$10,000,000 total across components; Component 1 (core) \$2,450,000. CONFIRM before submit.

Abstract

FAIRBANKS MEDICAL CENTRE LIMITED (FairBanks) is a health organisation in Kampala, Uganda. We run FairBanks Medical Centre and FairBanks Community Reach. Our CHWs and VHTs already work in Bukoto, Kyebando, Kisaasi, Kamwokya, Kikaaya and nearby communities. We meet people in homes, schools, and local gatherings, not only at the clinic. Our slogan is simple: Your health, our mission. Uganda has made progress on health security, but the 2023 Joint External Evaluation still pointed to real gaps. Community and clinic reports often reach decision makers too late. Information does not always move well between sectors. Subnational teams need more workers ready for surge response. Links into national surveillance, including the National Integrated Surveillance System (NISS), are still incomplete. When that happens, outbreaks grow before anyone can act. Ugandans feel the harm first. Americans are safer when threats are stopped closer to where they start. This project helps close that last mile gap under Ministry of Health leadership. We are not trying to replace national systems. We want community and facility signals to reach MoH pathways faster and in a form government can use. In Year 1 (Component 1, Core GHS) we will focus on four practical things: (1) community and facility surveillance that feeds MoH and NISS pathways using our FCHIP tools; (2) faster detect, notify, and respond timing with districts, using CDC 717 style benchmarks; (3) training CHWs, VHTs, and frontline staff for surveillance and surge readiness, including One Health awareness; and (4) stronger links between communities and facilities for priority diseases such as HIV, TB, malaria, cholera, viral hemorrhagic fevers, mpox, and immunisation work. For laboratory and border health, we will support MoH aligned partners rather than claim a national lead role alone. Components 2 to 5 are contingency plans for small and large outbreaks, emerging threats, and humanitarian emergencies. CDC may approve them but leave them unfunded until emergency money is released. If we succeed, alerts should move faster, community reports should be more complete, frontline workers should be better prepared, and tools and data should be handed to government for continued use. Contact: Racheal Nabukeera, info@fairbanksmedicalcentre.org, +256772849258.